

StrokeGuard

Clinical Decision Support System | Doctor-guided decisions come first

Important: This summary provides an experimental estimate based only on the information entered. Do not rely on it alone for diagnosis, treatment, emergency decisions, fitness, or medical clearance. A qualified doctor's assessment and advice should remain the primary basis for care.

Assessment summary	Result
Symptom-based indicator	50.3%
Assessment range	Moderate indicator range
Pattern summary	Elevated pattern identified
Selected symptoms	3
Assessment time	31 July 2026, 04:23 PM

Patient inputs

Parameter	Recorded value
Age	43
Gender	Female
Chest pain	Present
Hypertension	Absent
Irregular heartbeat	Absent
Shortness of breath	Absent
Fatigue or weakness	Present
Dizziness	Absent
Edema or swelling	Absent
Neck or jaw pain	Absent
Excessive sweating	Absent
Persistent cough	Absent
Nausea or vomiting	Absent
Chest discomfort	Absent
Cold hands or feet	Absent
Sleep apnea	Absent
Anxiety	Present

Indicator insights

These values show how the estimate changed when one selected input was altered while the other inputs stayed the same. They do not prove cause, severity, or an individual medical contribution.

Input comparison	Score change
Chest pain: Compared with chest pain set to absent	+50.3 points
Fatigue or weakness: Compared with fatigue or weakness set to absent	+50.2 points
Age: Compared with reference age 39	+49.9 points
Anxiety: Compared with anxiety set to absent	+48.4 points

Guidance to discuss

Do not ignore new or severe symptoms

New, severe, persistent, or worsening chest or breathing symptoms require prompt professional assessment.

Separate anxiety from physical warning signs

Anxiety can overlap with physical symptoms, but sudden or severe neurological symptoms should still receive urgent medical attention.

Know the BE FAST warning signs

Sudden balance loss, vision change, facial droop, arm weakness, or speech difficulty requires emergency action.

Support long-term vascular health

Follow clinician-approved plans for activity, sleep, diet, smoking avoidance, and management of blood pressure, glucose, and lipids.

Possible clinical discussion topics

Topic	Why it may be discussed
Cardiovascular assessment	The appropriate evaluation depends on onset, severity, and history.
Glucose review	Discuss screening frequency based on individual risk.
Lipid profile	Review cholesterol and broader vascular risk when appropriate.
Focused clinical examination	A professional examination provides essential context beyond the entered information.